Document Control

Title

Neonatal Endotracheal Suction Standard Operating Procedure

Author's job title
Community Respiratory physiotherapist

Directorate	**Department**	**Team/Specialty**
Health and Social Care Community Services	Respiratory	CREADO

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0.1	Jul 2015	Draft	Initial version for consultation
1.0	Jul 2015	Final	Approved by the Respiratory Therapy Group. Transferred to new Trust forma at published on the Website
2.0	April 2019	Final	Approved by the Respiratory Therapy Group

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Senior Management	Trust's website

Superseded Documents

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Consulted with the following stakeholders:

- Ward Managers
- Physiotherapy In Patient Manager
- Physiotherapy Advanced Practitioner
- Infection Control Team
- Assistant Director of Nursing

Approval and Review Process

- Respiratory Physiotherapy Specialist Interest Group
- Assistant Director of Nursing

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Respiratory, Physiotherapy, CREADO	Respiratory, Suction, Airway

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1. Introduction

This document sets out Northern Devon Healthcare NHS Trust's procedure for the use of Neonatal Endotracheal Suction. It provides a clear process to ensure a consistent approach across all clinical areas within the Trust.

2. Purpose

The Standard Operating Procedure (SOP) has been written to:

- Support the main Suction Policy and should be used in conjunction with this.
- Identify the procedure to be implemented when the use of Neonatal Endotracheal Suction is required. Please note that this is a guideline only and a clinician's clinical judgement should be utilised following a thorough assessment of the patient.

3. Scope

This Standard Operating Procedure (SOP) relates to the following staff groups who may be involved in the assessment and delivery of neonatal endotracheal suction on the wards:

- Registered nurses
- Physiotherapists
- Medical staff

4. Location

This Standard Operating Procedure ~ Neonatal Endotracheal Suction can be implemented in all clinical areas where competent staffs are available to undertake this role.

Staff undertaking this procedure must be able to demonstrate continued competence as per the organisations policy on assessing and maintaining competence.

5. Equipment

- Functional suction unit pressures set 60-80mmHg/ 8-11 Kpa
- Appropriately sized suction catheters with port incorporated or the addition of a port
- Patient 6' (ft) tubing
- Sterile water and jug
- Personal protective equipment

- Neopuff
- Observation machine
- Sterile gloves
- Tape measure
- Stethoscope

6. Procedure

- Assess the need for suction. Avoid suction for 24 hours after surfactant administration unless in an emergency and the endotracheal tube has become blocked.
- This procedure should be performed as an aseptic technique.
- Explain the procedure if parents are present and gain consent.
- Wash your hands and don appropriate personal protective equipment.
- Position head in the mid-line if possible to reduce changes in cerebral blood flow.
- Establish length of the endotracheal tube (including dead space and length of blue ETT hub). This is the length to pass the suction catheter to ensure that the catheter reaches the end of the ETT.
- Do not pre oxygenate unless instructed to by the paediatrician.
- Monitor the observations before pre and post procedure.
- Set the suction pressure. Turn on the suction at the unit/ machine. Ensure the patient tubing is attached to the suction unit/ machine. Occlude the end of the patient tubing with your thumb. Observe the dial on the suction unit, adjust the pressure using the pressure regulator knob to 60-80mmHg or 8-11Kpa.
- Select the catheter size by doubling the size of the endotracheal tube
- Open the end of the catheter packaging.
- Attach the catheter whilst contained within the packaging, to the end of the patient tubing incorporating a port if there isn't one within the suction catheter.
- Have the neopuff close to hand and set the same amount of oxygenation as the ventilator, in case of a drop in oxygen saturations post suction.
- Using a sterile glove on your dominant hand remove the suction catheter from its packaging.
- The time taken to pass and withdraw a suction catheter from the ETT should ideally be between 5-8 seconds or less for neonates.
- Insert the catheter using your sterile hand to the pre-determined depth, using the graduations on the catheter.
- Apply suction on withdrawal by covering a thumb over the port for no longer than 10 seconds. Do not rotate the catheter.
- Reattach the ventilator.
- Once the catheter is fully withdrawn from the airway, discontinue suction and wind the catheter around the gloved hand, remove the glove over catheter. Discard the glove and catheter together, according to local infection control policy.
- Flush through the suction tubing using sterile water in a jug.

7. References

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- Hough, A (2001) Physiotherapy in Respiratory Care. Third Edition. Nelson Thomes Ltd: Glos. p207-208

8. Associated Documentation

Northern Devon Healthcare NHS Trust Policies for :

- Suction Policy
- Single Use Policy
- Consent Policy
- Aseptic Techniques Policy
- Mental Capacity Act Policy
- Standard Infection Control Precautions Policy